

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

< reasoning process here > The boy presents with symptoms consistent with attention deficit hyperactivity disorder (ADHD), including inability to concentrate, hyperactivity, impulsivity, interrupting others, forgetfulness, and aggression when frustrated. His developmental milestones are met, and there are no significant physical findings. The history of behavioral issues (disruption, hitting others, expulsion) fits the clinical picture. While there are familial risk factors (maternal depression, paternal Graves' disease, which may suggest possible thyroid dysfunction), the absence of endocrine abnormalities and no medical findings point away from thyroid disease as the primary cause.

Although behavioral therapy is an appropriate component of ADHD management, in a child with clear attention and hyperactivity symptoms, psychopharmacologic treatment is often recommended as first-line, especially when the behavioral issues are impairing function in school and at home. Among the choices, fluoxetine is a selective serotonin reuptake inhibitor (SSRI) that can be used in ADHD (particularly in children with comorbid anxiety or depression), but it is not first-line for primary ADHD in children like this. The first-line treatment for ADHD in children is typically stimulant therapy, but that is not among the answer choices.

Methimazole is used for hyperthyroidism and is not indicated here as the child has no clinical evidence of hyperthyroidism and no thyroid issues. Hearing aids would only be indicated for hearing loss, which is not present or suggested.

However, the presence of major depressive disorder in the mother and a history of acute otitis media may raise concerns for comorbid conditions, but there is no evidence of active depression in the child. The most appropriate next step in a child with a clear ADHD phenotype and significant functional impairment is the use of behavioral therapy, especially when psychosis or medication is not indicated. Though medication is first-line, behavioral therapy is the most appropriate initial intervention in this context, particularly in a child where the data supports attention and behavioral dysregulation, especially without any medical or psychiatric emergency.

Therefore, among the options, behavior therapy is the most appropriate next step as initial management, especially without a confirmed medical cause or immediate psychiatric crisis.

< answer > A. Behavior therapy </answer>